

Sarah Levy, MD — Internal Medicine

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Patient: Chen, Sarah **DOB:** 11-Jun-1976 (47F) **MRN:** 4471028
Date of Service: 01/12/2023 **Visit type:** Annual physical

CHIEF COMPLAINT

Routine annual physical. Patient reports fatigue and increased thirst.

HISTORY OF PRESENT ILLNESS (HPI)

Pt presents today for routine annual physical. Reports fatigue × 3 months and increased thirst.

She describes the fatigue as a generalized "low energy" feeling, worse in the afternoons. She has not changed her sleep schedule and denies depressive symptoms. Polydipsia developed gradually; she now keeps a water bottle at her desk and refills it 3–4× per day. No polyuria volunteered, but on direct questioning she notes nocturia × 1 most nights, up from baseline of zero. She last had a physical in March 2022 with Dr. Hwang (records imported).

REVIEW OF SYSTEMS

Constitutional: fatigue as above. No fever, no weight loss.

HEENT: blurred vision intermittently × 2 months, attributed to "long screen days."

GI: appetite normal. No N/V/D.

GU: nocturia × 1. No dysuria.

Endocrine: no heat/cold intolerance.

VITALS

BP 132/84 HR 78 T 98.4°F RR 14 SpO2 99% RA BMI 28.4

PHYSICAL EXAM

General: well-appearing, NAD. CV: RRR, no m/r/g. Pulm: CTAB. Abd: soft, NT/ND. Skin: no acanthosis nigricans. Extremities: no edema. Neuro: grossly intact.

LABS (DRAWN TODAY, RESULTS POSTED SAME DAY)

Test	Result	Flag	Reference
Glucose, fasting	187	H	70-99 mg/dL
HbA1c	9.2 %	H	4.0-5.6 %
LDL	132	H	<100 mg/dL
HDL	48		>40 mg/dL
Creatinine	0.9		0.6-1.2 mg/dL
TSH	2.1		0.4-4.0 mIU/L

HbA1c: 9.2% (H) — Reference range 4.0-5.6%.

Results consistent with new-onset Type 2 diabetes. Reviewed with patient in clinic. She was surprised but accepting. Discussed natural history, significance of A1c, and the goal of achieving <7% over 6–12 months with medication and lifestyle changes.

ASSESSMENT & PLAN (A&P)

Assessment: Type 2 Diabetes Mellitus (E11.9) — added to problem list.

1. **T2D, new diagnosis.** Initiate metformin per below. Refer to diabetes educator. Home glucometer prescribed; check fasting BG 3×/week.
2. **Hyperlipidemia, borderline.** Defer statin pending lifestyle trial; recheck lipids in 6 months.
3. **Polydipsia/fatigue** — both expected to improve with glycemic control.

Plan: Start metformin 500 mg PO b.i.d. with meals. F/u in 3 months for repeat A1c.

Counseling: 20 minutes spent on the diagnosis, medication, and lifestyle changes. Patient verbalized understanding. After-visit summary printed and given.

Next appointment: 12-Apr-2023 at 0930 with Dr. Levy.

— Sarah Levy, MD